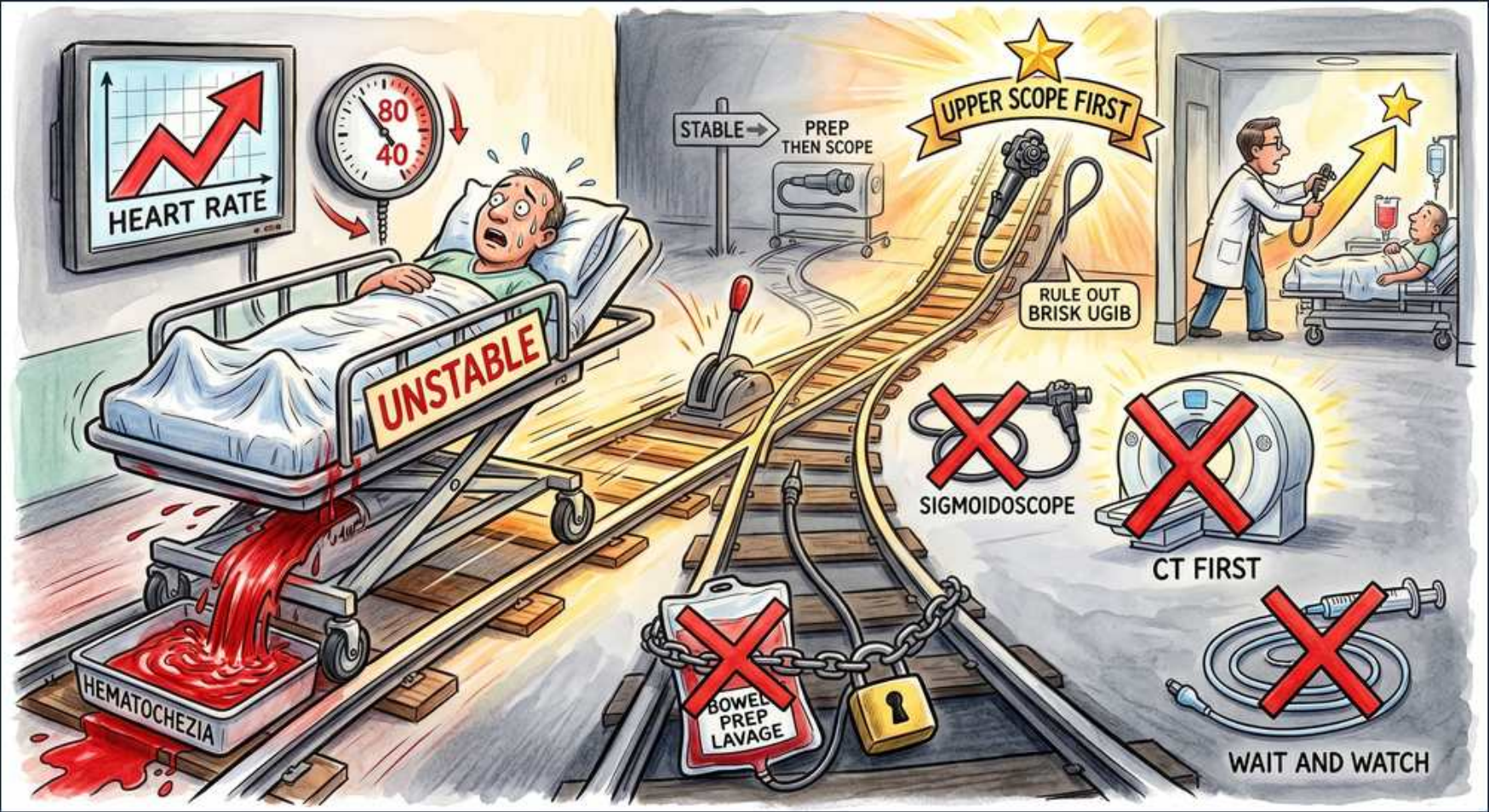


GI Bleed — Brisk Hematochezia Pathway



1. UNSTABLE patient

WHAT YOU SEE

Sweating patient on a gurney with a red 'UNSTABLE' label on the bed rail.

WHAT IT ENCODES

Hemodynamic instability = the branch point. Unstable means SBP <90, HR >100, orthostatic drop, or ongoing brisk bleeding despite resuscitation. Instability overrides the 'bright red blood = lower' instinct and changes the first test.

2. Heart rate up / BP down

WHAT YOU SEE

Monitor 'HEART RATE' with a rising red arrow and a gauge showing 80/40.

WHAT IT ENCODES

Tachycardia with hypotension (here 80/40) marks significant blood loss. Tachycardia is the earliest vital sign change; hypotension implies >30% volume loss. These numbers define 'unstable' and trigger upper scope first.

3. Hematochezia (brisk)

WHAT YOU SEE

Bright red blood pooling in a basin labeled 'HEMATOCHEZIA' under the gurney.

WHAT IT ENCODES

Bright red blood per rectum. Although usually a lower source, ~10-15% of UPPER GI bleeds present this way when bleeding is brisk enough to transit fast. In an unstable patient, a massive upper bleed must be excluded first.

4. UPPER SCOPE FIRST (gold star)

WHAT YOU SEE

Gold-star banner 'UPPER SCOPE FIRST' over the railway switch.

WHAT IT ENCODES

THE rule: unstable + hematochezia = EGD (upper endoscopy) FIRST to rule out a brisk upper GI bleed. EGD lets you find and treat the source in one step. Only after a negative EGD do you pursue the colon.

5. Rule out brisk UGIB

WHAT YOU SEE

Label 'RULE OUT BRISK UGIB' on the track toward the doctor scoping.

WHAT IT ENCODES

The purpose of upper-first is to exclude a brisk upper GI bleed presenting as hematochezia. Up to 15% of these patients have an upper source; a sigmoidoscope or colonoscope would miss it entirely.

6. Doctor doing EGD (gold star)

WHAT YOU SEE

Physician performing upper endoscopy on a calm patient, gold star above.

WHAT IT ENCODES

EGD performed urgently after resuscitation. If EGD is negative and bleeding continues, proceed to colonoscopy (after prep if it can be done) or angiography/tagged-RBC scan for very brisk ongoing bleeds.

7. Stable -> prep then scope

WHAT YOU SEE

Gray side-track 'STABLE -> PREP THEN SCOPE' leading to a small endoscope.

WHAT IT ENCODES

If the patient is hemodynamically STABLE, the pathway is bowel prep (polyethylene glycol) then COLONOSCOPY, the test of choice for lower GI bleeding, ideally within 24h. Stability is what makes prep-and-colonoscopy safe.

8. Sigmoidoscope first (RED X)

WHAT YOU SEE

Endoscope with a red X labeled 'SIGMOIDOSCOPE'.

WHAT IT ENCODES

Flexible sigmoidoscopy first in an unstable hematochezia patient is wrong: it visualizes only the rectosigmoid, misses the right colon and any upper source. It is not the diagnostic move in brisk unstable bleeding.

BOARD TRAP

'flexible sigmoidoscopy first' -> wrong; it misses the right colon and upper sources; scope the top first when unstable.

9. Colonoscopy/bowel prep first (RED X)

WHAT YOU SEE

Bowel-prep jug padlocked with a red X labeled 'BOWEL PREP / LAVAGE'.

WHAT IT ENCODES

Trying to prep and colonoscope an UNSTABLE patient is wrong: prep takes hours, risks aspiration and further destabilization, and may still miss a brisk upper source. Stabilize and scope the top first.

BOARD TRAP

'prep then colonoscopy first' in an unstable patient -> wrong; too slow, aspiration risk, misses upper source.

10. CT angiography first (RED X)

WHAT YOU SEE

CT scanner with a red X labeled 'CT FIRST'.

WHAT IT ENCODES

CT angiography first is wrong as the initial move in unstable hematochezia: it is purely diagnostic (no therapy), needs active bleeding $\geq 0.3\text{--}0.5$ mL/min to localize, and delays the definitive EGD. It is a later option if EGD/colonoscopy fail.

BOARD TRAP

'CT angiography first' -> wrong; non-therapeutic, needs active brisk bleeding, delays the diagnostic-therapeutic EGD.

11. Wait and watch (RED X)

WHAT YOU SEE

Syringe/observation icon with red X labeled 'WAIT AND WATCH'.

WHAT IT ENCODES

Observation / 'wait and watch' is wrong for an unstable bleeding patient: ongoing hemorrhage with shock demands immediate resuscitation and EGD, not delay. Passive management lets the patient exsanguinate.

BOARD TRAP

'wait and watch / observe' in unstable hematochezia -> wrong; needs urgent resuscitation and upper endoscopy.
